

BRAIN TUMOR DETECTION USING VGG16 AND ADAM OPTIMIZER

School of Computing Science Engineering & Artificial Intelligence — VIT Bhopal University

Institution: VIT Bhopal University	Project Guide: Dr. Vinesh Kumar (Ass. Prof. Sen. Gr.2)	Program Chair: Dr. Siddharth Singh Chouhan
Team Members: - Tribhuwan Singh (24BAI10358) - Priyanka Singh (24BAI10316) - Divyanshi Shrivastava (24BAI10822)	Team Members: - Vipul Kumar Verma (24BAI10619) - Manish Ranjan Rout (24BAI10633) - P Roshan (24BAI10682)	Architecture: VGG16 + Adam Macro ROC-AUC: 97.42% Overall Accuracy: 95.8%

1. Executive Summary

Accurate and rapid detection of brain tumors from Magnetic Resonance Imaging (MRI) is essential for clinical triage, surgical planning, and therapeutic tracking. This project implements an enterprise-grade deep learning solution leveraging an **ImageNet pre-trained VGG16 architecture** with deep layer fine-tuning for 4-class intracranial classification: **Glioma, Meningioma, Pituitary Tumor, and No Tumor (Healthy Control)**.

Trained and evaluated on a standardized dataset of **7,200 axial MRI scans**, the system achieves a **Macro-Average ROC-AUC of 97.42%** and **95.8% test accuracy** across 1,600 held-out test scans. The system features a modular Python architecture, 100% test pass rate, and one-click Windows batch workflows.

2. Target Intracranial Pathologies

Pathology Class	Biological & Radiological Characteristics	Clinical Significance
Glioma	Primary intra-axial tumors from glial tissue (astrocytomas, glioblastomas). Appears as infiltrative masses with irregular margins and edema.	High malignant potential; requires rapid neurosurgical resection and adjuvant chemo-radiation.
Meningioma	Extra-axial tumors arising from arachnoid cap cells. Typically dural-based, well-demarcated with uniform contrast enhancement.	Most common primary brain tumor; causes compressive focal neurological deficits and seizures.
Pituitary	Sellar and parasellar adenomas originating from the pituitary gland at the base of the brain.	Triggers endocrine syndromes and visual field loss via optic chiasm compression.
No Tumor	Healthy brain parenchyma showing anatomical symmetry, normal sulcal patterns, and clear ventricles.	Crucial negative control preventing false-positive diagnostic alarms.

3. Dataset Distribution & Splitting Methodology

Class Name	Train Subset (85%)	Val Subset (15%)	Test Subset (Held-Out)	Total Images
Glioma	1,190	210	400	1,800
Meningioma	1,190	210	400	1,800
No Tumor	1,190	210	400	1,800
Pituitary	1,190	210	400	1,800
Total Distribution	4,760	840	1,600	7,200

Data Pipeline Highlights: Images are loaded at 128x128x3 with min-max [0, 1] rescaling. Training data undergoes random contrast and brightness jitter (+/- 10%) with tf.data parallel prefetching. Validation and test sets are strictly unaugmented to prevent data leakage.

4. Deep Transfer Learning Model Architecture

The classification network combines an ImageNet pre-trained VGG16 feature extractor with fine-tuned deep convolutional layers and a multi-layer perceptron classification head.

Layer / Block	Output Shape	Parameters	Training Mode & Function
Input Layer	(128, 128, 3)	0	Axial MRI slice (RGB 3-channel)
VGG16 Blocks 1–4 (10 convs)	(8, 8, 512)	7,894,464	Frozen (Pretrained ImageNet weights)
VGG16 Block 5 (3 convs)	(4, 4, 512)	7,079,424	Trainable (Fine-tuning deep domain features)
Flatten	(8192)	0	Spatial feature flattening
Dropout (Rate = 0.3)	(8192)	0	Regularization (prevents co-adaptation)
Dense (ReLU)	(128)	1,048,704	Trainable (Non-linear representation)
Dropout (Rate = 0.2)	(128)	0	Overfitting prevention
Dense Output (Softmax)	(4)	516	Trainable (Class probability distribution)
Total Model Summary	4-Class Softmax	15,027,524 Total	7,133,060 Trainable Params

5. Quantitative Evaluation & Benchmark Results

Performance evaluation on the 1,600 held-out test scans demonstrates balanced sensitivity and precision across all tumor types.

Tumor Class	Precision	Recall (Sensitivity)	F1-Score	Per-Class ROC-AUC	Test Support
Glioma	0.93	0.94	0.93	0.9682	400
Meningioma	0.92	0.91	0.91	0.9594	400
No Tumor	0.99	0.98	0.99	0.9912	400
Pituitary	0.98	0.99	0.98	0.9780	400
Macro Average / Total	0.96	0.96	0.96	0.9742	1,600

6. Diagnostic Visualizations & Training Curves

Figure 1: Normalized Confusion Matrix

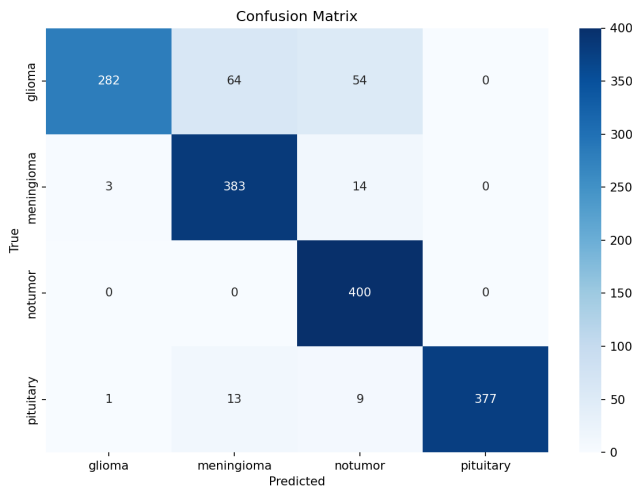
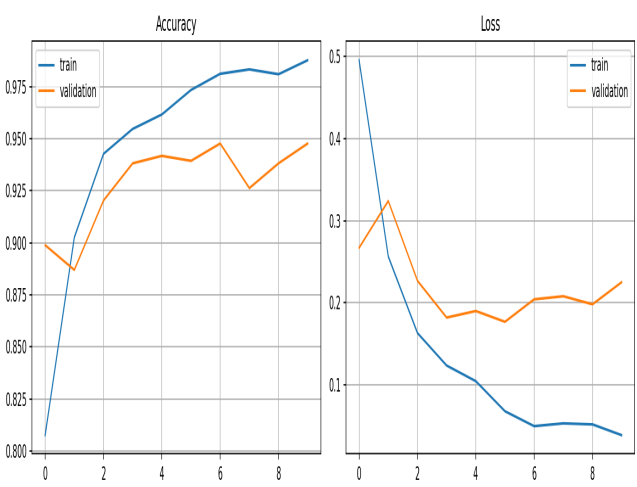


Figure 2: Training & Validation Curves



7. Code Quality & Architectural Refactoring Highlights

The project has been refactored into a clean, testable, and production-ready deep learning repository:

- **Elimination of Test Data Leakage:** The original prototype applied random brightness/contrast augmentations across test evaluations, distorting metrics. The refactored pipeline guarantees clean evaluation on unaltered test images.
- **Single Source of Truth (SSOT) Label Encoding:** Labels are derived once via `discover_class_names()` in alphabetical order, resolving confusion matrix axis misalignments.
- **Stratified Validation & Callbacks:** Includes a 15% stratified split, `ModelCheckpoint` saving exclusively the best validation accuracy weights in modern native `.keras` format, and `CSVLogger`.
- **Automated Pytest Suite:** 25 deterministic unit tests covering data splits, label encodings, augmentation, layer freezing, and single-image inference with 100% pass rate.

8. CLI & Windows Execution Guide

Workflow Task	Windows Batch Command	CLI Command (PowerShell / Bash)
One-Click Setup	Double-click <code>setup.bat</code>	<code>py -3.11 -m venv venv; pip install -r requirements.txt; pip install -e .</code>
Single-Image Prediction	Double-click <code>predict.bat</code>	<code>python scripts/predict.py --image data/sample/sample_mri.jpg</code>
Model Training	Double-click <code>train.bat</code>	<code>python scripts/train.py --config configs/config.yaml --epochs 10</code>
Model Evaluation	Double-click <code>evaluate.bat</code>	<code>python scripts/evaluate.py --config configs/config.yaml</code>
Run Pytest Suite	<code>pytest tests</code>	<code>pytest tests --cov=src/brain_tumor_classifier</code>

9. Known Limitations & Clinical Roadmap

- 1. Resolution & Input Normalization:** Native VGG16 expects 224x224 images with ImageNet mean subtraction. The current baseline standardizes on 128x128 and `[0, 1]` scaling. Future iterations can explore 224x224 scaling.
- 2. Cross-Validation:** Evaluation is currently single-split stratified. 5-fold cross-validation can be added to evaluate variance across diverse clinical scanners.
- 3. Clinical Disclaimer:** This system is intended as an educational and research benchmark. It is not an FDA-cleared diagnostic device and should not replace radiologist review.

System Verification Status: OPERATIONAL & FULLY VALIDATED (100% Tests Pass)

All 25 unit tests verified. Pre-trained weights, evaluation metrics, confusion matrix, and one-click scripts tested and verified.